

T - OMESEC 20MG CAP N.3 [FRONT]



OmesecTM
20 mg Capsules



PRODUCT DESCRIPTION
Hard gelatin capsule with pink body and caramel cap, each half printed with 'OMESEC 20' in black, holding enteric-coated mini-tablets.

COMPOSITION
Active Ingredient : Omeprazole 20 mg/capsule
Preservatives : None

PHARMACODYNAMICS
Omeprazole is a proton-pump inhibitor and thus prevents the secretion of gastric acid. Omeprazole is a weak base and is activated in acidic pH to a sulphonamide derivative that binds irreversibly to Hydrogen/Potassium Adenosine Triphosphatase (H⁺/K⁺ ATPase) enzyme system, the so called 'proton pump' that is found at the secretory surface of parietal cells. It thereby inhibits the final transport of hydrogen ions (via exchange with potassium ions) into the gastric lumen. Omeprazole inhibits both basal and stimulated acid secretion irrespective of the stimulus. It also inhibits hepatic cytochrome P-450 mixed function oxidase system.

During treatment with antisecretory medicinal products, serum gastrin increases in response to the decreased acid secretion. Also CgA increases due to decreased gastric acidity. The increased CgA level may interfere with investigations for neuroendocrine tumours.

Available published evidence suggests that proton pump inhibitors should be discontinued between 5 days and 2 weeks prior to CgA measurements. This is to allow CgA levels that might be spuriously elevated following PPI treatment to return to reference range.

PHARMACOKINETICS
Omeprazole is rapidly absorbed, but to a variable extent following oral administration. The absorption of Omeprazole is formulation-dependent and dose-dependent but it is not affected by food. Omeprazole in the form of enteric-coated mini-tablets is absorbed from the small intestines and delivered via the bloodstream to the parietal cell. Oral absorption of the enteric-coated formulation is relatively slow and may be further influenced as a result of delayed gastric emptying.

Omeprazole is distributed in tissue, particularly gastric parietal cells. It is highly bound to plasma proteins, approximately 95% bound to albumin and alpha1-acid glycoprotein. The onset of action is within one hour and the time to peak effect is within 2 hours. It has a duration of action of up to 72 hours or more. In a normal hepatic function, the plasma half-life is 30 minutes to 1 hour while in a chronic hepatic disease, the plasma half-life is 3 hours.

Omeprazole is almost completely metabolised in the liver. The three major metabolites identifiable in plasma are hydroxyomeprazole, a sulfide derivative and a sulfone. Omeprazole is subject to an important polymorphism of drug metabolism as it is a substrate for CYP2C19, the S-mephenytoin hydroxylase.

Omeprazole is rapidly eliminated, mostly in the urine as metabolites. Some is excreted in the faeces. The bioavailability of Omeprazole may be increased in elderly patients and in patients with impaired hepatic function, but is not markedly affected in patients with renal impairment.

INDICATIONS
Omeprazole is used in the treatment of gastro-oesophageal reflux disease; duodenal ulcer, benign gastric ulcer and Zollinger-Ellison Syndrome. It is also combined with antibacterial agents in dual or triple therapy regimens for the elimination of *Helicobacter Pylori* infection in patients with duodenal ulcer.

SIDE EFFECTS/ADVERSE REACTIONS
Skin and subcutaneous tissue disorders
Skin rash, urticaria and pruritus have been reported, usually resolving after discontinuation of treatment.

Frequency 'not known': Subacute cutaneous lupus erythematosus.

Central and peripheral nervous system
Headache is a common side effect. Dizziness, paraesthesia, somnolence, insomnia and vertigo are rare. In isolated cases, reversible mental confusion, agitation, depression and hallucinations predominantly in severely ill patients have occurred. Increased sweating, peripheral oedema and blurred vision have been reported in isolated cases.

Gastrointestinal Disorder
Diarrhoea, constipation, nausea, vomiting, flatulence and abdominal pain. Frequency 'common': Fundic gland polyps (benign). Frequency 'not known': Microscopic Colitis



Hepatic
Rarely increased in liver enzymes. In isolated cases, encephalopathy in patients with pre-existing severe liver disease; hepatitis with or without jaundice and hepatic failure have been reported.

Endocrine
In isolated cases, gynaecomastia has been reported.

Haematological
In isolated cases, leukopenia, thrombocytopenia, agranulocytosis and pancytopenia have been reported.

Musculoskeletal
In isolated cases, arthralgia, muscular weakness and myalgia have been reported.
Frequency "uncommon": Fracture of the hip, wrist or spine

Renal and urinary disorders
Interstitial nephritis

Metabolism and nutritional disorders
Vitamin B12 deficiency
Frequency "not known": hypomagnesaemia

Infection & infestations
Clostridium difficile associated diarrhoea.

PRECAUTIONS/WARNINGS

Pregnancy
Adequate and well-controlled studies in humans have not been done. Studies in pregnant rats did not show Omeprazole to have any teratogenic potential at doses 345 times the human dose. Avoid in pregnancy unless there is no safer alternative.

Breast-feeding
It is not known whether Omeprazole is distributed into human milk. However, because Omeprazole has been shown to cause tumourigenic and carcinogenic effects in animals, a decision should be made on whether breast feeding should be discontinued or the medication withdrawn, taking into account the importance of the Omeprazole to the mother.

Geriatrics
A decreased rate of elimination and an increased bioavailability are likely to occur in geriatric patients taking Omeprazole.

Gastric Ulcers
Before giving Omeprazole to patients with gastric ulcers, the possibility of malignancy should be excluded since Omeprazole may mask symptoms and delay diagnosis.

Hepatic Disease
In chronic hepatic disease, dosage reduction may be required due to increased half-life.

Hypomagnesaemia
Severe hypomagnesaemia has been reported in patients treated with PPI like Omesec for at least three months, and in most cases for a year. Serious manifestations of hypomagnesaemia such as fatigue, tetany, delirium, convulsions, dizziness and ventricular arrhythmia can occur but they may begin insidiously and be overlooked. In most affected patients, hypomagnesaemia improved after magnesium replacement and discontinuation of the PPI.

For patients expected to be on prolonged treatment or who take PPI with digoxin or drugs that may cause hypomagnesaemia (e.g., diuretics), health care professionals should consider measuring magnesium levels before starting PPI treatment and periodically during treatment.

Vitamin B12 Deficiency
Daily treatment with any acid-suppressing medications over a long period of time (e.g., longer than 3 years) may lead to malabsorption of cyanocobalamin (vitamin B12) caused by hypo- or achlorhydria. Rare reports of cyanocobalamin deficiency occurring with acid-suppressing therapy have been reported in the literature. This diagnosis should be considered if clinical symptoms consistent with cyanocobalamin deficiency are observed.

Clostridium Difficile Diarrhoea
Published observational studies suggest that PPI therapy may be associated with an increased risk of Clostridium difficile associated diarrhea (CDAD), especially in hospitalized patients. This diagnosis should be considered for diarrhea that does not improve. Patients should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated. CDAD has been reported with use of nearly all antibacterial agents. For more information specific to antibacterial agents (clarithromycin and amoxicillin) indicated for use in combination with Omesec Capsule, refer to Warnings and Precautions sections of those package inserts.

Regular Surveillance
Patients on proton pump inhibitor treatment (particularly those treated for long term) should be kept under regular surveillance.



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Fracture

Proton pump inhibitors, especially if used in high doses and over long durations (>1 year), may modestly increase the risk of hip, wrist and spine fracture, predominantly in the elderly or in presence of other recognised risk factors. Observational studies suggest that proton pump inhibitors may increase the overall risk of fracture by 10-40%. Some of this increase may be due to other risk factors. Patients at risk of osteoporosis should receive care according to current clinical guidelines and they should have an adequate intake of vitamin D and calcium. Patient should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated.

Subacute Cutaneous Lupus Erythematosus (SCLE)

Proton pump inhibitors are associated with very infrequent cases of subacute cutaneous lupus erythematosus (SCLE). If lesions occur, especially in sun-exposed areas of the skin, and if accompanied by arthralgia, the patient should seek medical help promptly and the health care professional should consider stopping Omesec capsule. SCLE after previous treatment with a proton pump inhibitor may increase the risk of SCLE with other proton pump inhibitors.

Concomitant use Omesec Capsule with methotrexate

Literature suggests that concomitant use of PPIs with methotrexate (primary at high dose; see methotrexate prescribing information) may elevate and prolong serum levels of methotrexate and/or its metabolite possibly leading to methotrexate toxicities. In high-dose methotrexate administration a temporary withdrawal of the PPI may be considered in some patients (See Drug Interactions).

Interference with laboratory tests

Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. If the patient(s) are due to have a test on Chromogranin A level, Omesec Capsule treatment should be stopped for at least 5 days before CgA measurements to avoid this interference (see section Pharmacodynamic). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.

CONTRAINDICATIONS

Omeprazole is contra-indicated in patients who are sensitive to it.

DRUG INTERACTIONS

Concurrent use of ampicillin esters, iron salts or ketoconazole with Omeprazole may result in a reduction in the absorption of these drugs due to the increased gastrointestinal pH by Omeprazole.

Omeprazole may also prevent the degradation of acid-labile drugs.

No interaction with food or concomitantly administered antacids has been found.

Plasma concentrations of Omeprazole and Clarithromycin are increased during concomitant administration.

Concurrent use of digoxin with Omeprazole may result in an increased in the plasma concentration of digoxin. Omeprazole may cause a decrease in the hepatic metabolism of diazepam, phenytoin and warfarin by inhibition of the hepatic cytochrome P-450 enzyme system, resulting in a delayed elimination and increased blood concentrations of these medications when used concurrently. Hence, the monitoring of blood concentrations or prothrombin time for patients receiving warfarin or phenytoin is recommended and a reduction of dosage may be necessary.

Concurrent use of Omeprazole with bone marrow depressants may increase the leukopenic and/or thrombocytopenic effects of both these medications. If concurrent use is required, close observation for toxic effects should be considered.

Methotrexate

Case reports, published population pharmacokinetics studies, and retrospective analyses suggest that concomitant administration of PPI and methotrexate (primarily at high dose; see methotrexate prescribing information) may elevate and prolong serum levels of methotrexate and/or its metabolite hydroxymethotrexate. However, no formal drug interactions studies of methotrexate with PPIs have been conducted (see Warnings and Precautions).

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION

Benign Gastric & Duodenal Ulcer : 20 mg once daily for 4 weeks in duodenal ulceration or 8 weeks in gastric ulceration. In severe or recurrent cases, the dose may be increased to 40 mg once daily.

Long term therapy for patients with a history of recurrent duodenal ulcer is recommended at a dosage of 20 mg once daily, up to one year.

For the prevention of relapse in patients with duodenal ulcer disease, the recommended dose is 10 mg once daily. If needed, the dose can be increased to 20-40 mg once daily.

Gastro-Oesophageal Reflux Disease : 20 to 40 mg once daily for 4 to 8 weeks; thereafter maintenance therapy can be continued with 10 mg once daily.

Zollinger-Ellison Syndrome : Initially 60 mg once daily; usual range 20 to 120 mg daily. Doses above 80 mg should be divided and given twice daily. The dosage should be adjusted individually and therapy continued for as long as clinically indicated.

Helicobacter Pylori Eradication:

Dual Therapy (2 Week Regimens)

A combination of Omeprazole 40 mg daily with Amoxicillin 750 mg twice daily or with Clarithromycin 500 mg three times daily for 2 weeks.

Triple Therapy (1-Week Regimens)

A combination of Omeprazole 20 mg twice daily or 40 mg once daily with Amoxicillin 500 mg 3 times daily and Metronidazole 400 mg 3 times daily for 7 days;

A combination of Omeprazole 20 mg twice daily or 40 mg once daily with Clarithromycin 250 mg twice daily and Metronidazole 400 mg (or Tinidazole 500 mg) twice daily for 7 days .

A combination of Omeprazole 20 mg twice daily or 40 mg once daily with Amoxicillin 1g twice daily and Clarithromycin 500 mg twice daily for 7 days.

Note : Dual therapy regimens have lower eradication rates than triple therapy regimens.

Paediatric

Dosage has not been established.

Geriatric

The daily doses usually should not exceed 20 mg.

Impaired hepatic function

As bioavailability and plasma half-life of Omeprazole are increased in patients with impaired hepatic function, a daily dose of 10 - 20 mg may be sufficient.

Route of Administration.

Omeprazole should be taken immediately before meals, preferably in the morning.

The capsule should be swallowed whole with liquid.

The contents of the capsule should not be chewed or crushed.

For patients with difficulties in swallowing, the capsule might be opened and the contents swallowed or suspended in a slightly acidic fluid e.g. juice, yoghurt or soured milk. The suspension should be taken within half an hour. Alternatively, the contents can be swallowed by sucking the capsule.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE(S)

The major clinical features are drowsiness, headache (possibly due to a metabolite) and tachycardia.

There is no specific antidote for overdosage with Omeprazole; the treatment should be symptomatic and supportive.

PHARMACEUTICAL INFORMATION

Storage Conditions

Store below 25°C in a cool dry place.

Protect from light and moisture.

Keep out of reach of children.

Jauh dari kanak-kanak.

Shelf Life

Refer to outer package.

Pack size/package

Omesec 20 mg Capsule: Blister packs of 2 x 7's, 4 x 7's, and 10 x 7's capsules.

Not all presentations may be available locally.

Gelatin for Capsule: Bovine source certified with Halal Certification

PRODUCT REGISTRATION HOLDER:

DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.

Lot No. 2, 4, 6, 8 & 10, Jalan P/7,

Section 13, Bangi Industrial Estate,

43650 Bandar Baru Bangi, Selangor, Malaysia.

MANUFACTURER:

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